



Revolutionizing Chronic Care

The Impact of VeeOne Health's 5G-Enabled Remote Patient Monitoring (RPM) on Hospital Readmissions and Clinical Efficiency

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Contact VeeOne Health (sales@veeonehealth.com) or T-Mobile Rep to review our full clinical data set and schedule a platform demonstration.

Executive Summary

As healthcare transitions from volume to value, the "Hospital-to-Home" model has become a strategic imperative. VeeOne Health, in partnership with T-Mobile, provides a turnkey Virtual Care-as-a-Service (VCaaS) ecosystem that extends clinical oversight beyond the hospital walls. By integrating the VeeOne OS with ultra-reliable 5G connectivity, health systems are achieving significant reductions in readmissions, enhancing patient adherence, and optimizing labor costs.

The Challenge: The High Cost of Readmissions

Hospital readmissions remain one of the most persistent financial and clinical burdens in U.S. healthcare. For chronic conditions such as Hypertension (HTN), Congestive Heart Failure (CHF), and COPD, the traditional post-discharge model often lacks the continuous data necessary to prevent acute exacerbations.

Data Driven Outcomes: The VeeOne Impact

Recent clinical evaluations and industry reports on high-performance RPM programs, including those utilizing the VeeOne infrastructure, demonstrate compelling results:

- **38% Reduction in All-Cause Admissions:** Large-scale deployments of 5G-enabled RPM have shown a nearly 40% drop in overall hospital admissions by catching physiological warning signs before they escalate into emergencies.
- **25% Enhancement in Patient Satisfaction:** The "plug-and-play" nature of the VeeGo 360 (pre-activated on T-Mobile's network) eliminates technical barriers, leading to higher HCAHPS scores and improved patient agency.
- **Significant Blood Pressure Control:** Clinical studies of EHR-integrated, team-based RPM programs (like the VeeOne model) have recorded average systolic blood pressure (SBP) reductions of 9.7 mmHg to 16.8 mmHg within the first 12 months for hypertension cohorts.
- **Reduced Length of Stay (ALOS):** By deploying "Hospital-at-Home" protocols, facilities can safely discharge patients earlier, knowing that continuous monitoring is active, effectively increasing bed capacity without physical expansion. In high-acuity virtual care models, this has resulted in an average reduction of 1.5 to 2 days in Length of Stay, as continuous monitoring provides the safety net required for earlier transition to home.

Technical Architecture: The 5G & OS Synergy

The success of the VeeOne solution is rooted in its **Single-Pane-of-Glass** architecture.

1. **Connectivity (The T-Mobile Advantage):** Unlike Wi-Fi-dependent systems, VeeOne devices arrive with built-in 5G. This ensures **zero-touch deployment**, meaning patients are "connected" the moment they open the box, drastically reducing the 15–20% churn rate typically caused by home connectivity issues.

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2. **Clinical Integration:** The platform integrates directly with major EHRs (Epic, Cerner), automating the flow of clinical notes and risk stratification metrics. This allows care managers to focus on intervention rather than data entry.
3. **Risk Stratification:** The system doesn't just collect data; it prioritizes it. By using advanced algorithms to flag deteriorating vitals, where RN's, MAs and/or clinicians can intervene with the highest-risk patients first and hence triaging to appropriate provider (MD or APP), optimizing staff productivity.

Economic Sustainability & 2026 Reimbursement

The 2026 CMS Final Rule has expanded the durability of RPM revenue. With the introduction of tiered management codes (e.g., **CPT 99470** for 10–19 minutes), programs are no longer "all-or-nothing."

- **CHF Reimbursement Capture:** Internal data from regional health systems demonstrates that a comprehensive, leveraging our technology-enabled care plan for **CHF was able to generate approximately \$1,400 in total reimbursement** per patient episode. This is achieved through the combination of RPM, CCM, and transition of care management codes.
- **Turnkey Profitability:** A standard 300-patient hypertension panel can generate over **\$30,000 in monthly gross revenue**, with a net profit for the clinic exceeding **\$16,000/month** even after platform and staffing fees.
- **Operational Offloading:** By utilizing outsourced clinical support through VeeOne's clinical ecosystem partners, hospitals can launch these programs with zero additional headcount, converting a fixed labor cost into a scalable, variable expense.

Ultimately, the synergy between high-speed 5G infrastructure and automated clinical risk stratification allows care teams to focus on high-impact interventions. By removing technical barriers for patients and operational hurdles for providers, health systems can deliver a superior care experience that is both clinically effective and economically resilient.

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